DPP DEPARTMENTAL POLICY AND PROCEDURE

TITLE:	EXTERNAL JUGULAR VEIN CANNULATION
DEPARTMENT:	INTENSIVE CARE UNIT
APPLIES TO:	ALL MEDICAL AND NURSING STAFF

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1. PURPOSE

1.1. To establish intravenous access in critically ill patient >8 years of age when unable who requires intravenous access for fluid or medication administration and in whom an extremity vein is not obtainable.

2. DEFINITION & ABBREVIATION

2.1. **External Jugular Vein** - receives the greater part of the blood from the exterior of the cranium and the deep parts of the face, being formed by the junction of the posterior division of the retro mandibular vein with the posterior auricular vein.

2.2. **Tourniquet** - used for stopping the flow of blood through a vein or artery, typically by compressing a limb with a cord or tight bandage.

2.3. **IV** - Intravenous

2.4. **PPE** - Personal Protective Equipment

2.5. **MCC**- Medica Cloud Care System (**HIS** – Hospital Information System)

3. RESPONSIBILITY

3.1. ICU Physician

3.1.1. Liable in performing the procedure with aseptic technique.

3.2. Staff Nurse

3.2.1. Must be trained and knowledgeable in assisting the cannulation of the external jugular vein

3.2.2. Must be informed IPC Department immediately if there is any infection will be noted.

3.3. Head Nurse

3.3.1. Must ensure that all staffs are competent and knowledgeable with the policy.

3.4. IPC Practitioner

3.4.1. Must be informed immediately if infection will be noted in the unit.

4. POLICY

4.1. There should be a complete physician's order in the MCC including the indication of external jugular vein cannulation.

4.2. Physician should be notified for any possible complications.

4.3. Maximum of two (2) attempts permitted using one side only due to possible airway compromise from the infiltration or hematoma formation at the site if the vessel is missed.

4.4. External Jugular Vein Access is a peripheral IV.

4.5. Don't use a pressure infuser in External Jugular site

4.6. Cannulation should not exceed more than 48 hours.

4.7. Strict compliance with IPC guidelines in standard precautions when performing the procedure. (Refer to MPP-IPC-0001 IPC Guidelines in Standard Precautions).

5. PROCEDURE

5.1. The ICU Physician should write a clear written order in the MCC.

5.2. Identify the patient correctly using the 2 identifiers. (Refer to APP-QPS-0016 Patient Identification).

5.3. Gather all equipment needed

5.4. Explain the procedure to the patient.

5.5. Provide privacy.

5.6. Place the patient in Trendelenburg position.

5.7. Perform hand hygiene and use PPE. (Refer to APPH-IPC-0005 Guidelines in Hand Hygiene).

5.8. Turn the patient's head toward the opposite side if no risk of cervical injury exists.

5.9. Look at both sides of the neck to find the largest external jugular vein.

5.10. Prepare the site as per peripheral IV site method with 2% Chlorhexidine/70% alcohol antiseptic solution.

5.11. Align the catheter with the vein and aim toward the same side shoulder

5.12. Tourniquet the vein lightly with one finger above the clavicle. Puncture the vein midway between the angle of the jaw and the clavicle and cannulate the vein in the usual method.

5.13. Confirm placement, attach the 3waystopcok and flush with distilled water.

5.14. Secure with Transparent Adhesive Tape (Opsite) and label with date, time and cannula gauge.

5.15. Monitor for any signs of air embolism, catheter embolism, hematoma or infiltration.

5.16. Inform the physician immediately if hematoma or infiltration noted.

5.17. Document the procedure in Medica Cloud Care (MCC) system and Infection Control Form.

6. MEASUREMENT

6.1. 100% of the medical and nursing staff are well oriented and adheres to this policy.

7. REFERENCES /CROSS- REFERENCES

7.1. https://www.merriam-webster.com/medical/tamponade

7.2. https://en.wikipedia.org/wiki/External_Jugular_Vein

7.3. https://www.ecu.edu/cs-dhs/ecuem/upload/0615_EMS_Protocols_3_of_4.pdf

7.4. MPP-IPC-0001 IPC Guidelines in Standard Precautions

7.5. APP-QPS-0016 Patient Identification

7.6. APPH-IPC-0005 Guidelines in Hand Hygiene

8. ATTACHMENT

8.1. External Jugular Vein Cannulation Competency Check Off

9. REVISION CLAUSE

9.1. Changes to the hospital policy and procedure template based from the Al-Baha Health Cluster logo, font face and design.

9.2. Modification to the definition, responsibility, policy and procedure contents.